

Obscura Overview

Private clinical AI on the clinic's own laptop

BUILT FOR / COMMUNITY CLINICS • SMALL & RURAL PRACTICES • FQHCs — AND EVERY HEALTH SYSTEM ALREADY PAYING FOR CLOUD TRANSCRIPTION

MARKET PROOF / EMORY • YALE NEW HAVEN • UNC HEALTH • CHRISTUS • COREWELL • U. KANSAS HEALTH SYSTEM +8 MORE ALREADY BUY CLOUD SCRIBES (ABRIDGE PUBLIC ROSTER) — SUKI COUNTS 300 SYSTEM & CLINIC CLIENTS

OBSCURA / SOFTWARE
FOUR TOOLS. ONE LAPTOP.
ZERO CLOUD.

LICENSE / APACHE-2.0
MODEL / GEMMA 4, LOCAL
DATA PATH / STAYS HOME

BUILT / JULY 2026
→ [GITHUB.COM/MEETKPATEL/OBSCURA](https://github.com/MEETKPATEL/OBSCURA)



Obscura →

Who This Is For

Obscura builds private AI for the clinics America runs on.

THE DESK TAX

2 hours of desk + EHR work per 1 hour of direct patient care. *Sinsky et al., Annals of Internal Medicine*

THE EXODUS

1 in 5 physicians intend to leave medicine within two years. *Mayo Clinic Proceedings survey*

THE EXPOSURE

\$9.8M average healthcare breach — costliest industry, 14 years running. *IBM Cost of a Data Breach, 2024*

THE PEOPLE

31M+ patients depend on community health centers on thin margins. *HRSA Health Center Program, 2023*



The market is already paying — just not for everyone.

CATEGORY TODAY	\$1.2–2.8B AI clinical documentation market, forecast ~\$15B by 2034–35 (19–29% CAGR across analyst houses).
THE INCUMBENTS	Abridge \$5.3B valuation · Suki \$165M raised, 300 system & clinic clients · Nuance DAX list price \$1,512/clinician/mo.
THE CEILING	All of it cloud. All of it priced for enterprise health systems with IT departments and BAA counsel.
THE OPENING	Most U.S. physicians work in practices of 10 or fewer (AMA benchmark) — the segment no cloud vendor prices for. That segment is ours, at \$0.

WHO ALREADY BUYS CLOUD TRANSCRIPTION — ABRIDGE'S PUBLIC CUSTOMER ROSTER

Emory Healthcare	Yale New Haven Health
UNC Health	CHRISTUS Health
Corewell Health	U. Kansas Health System
Rochester Regional	Cambridge Health Alliance
Akron Children’s	Lee Health
Riverside Health	Reid Health
Tanner Health	U. Vermont Health

Every name above signed with a cloud vendor. Every clinic below their size threshold could not — until the tooling became free and local.

Obscura →

What is needed for
private clinical AI?

How does AI reach the exam room safely?

The clinic's
reality

Visit audio

Patient documents

A messy file system

One aging laptop



?

The clinician's
need

Signed notes

Shareable, safe records

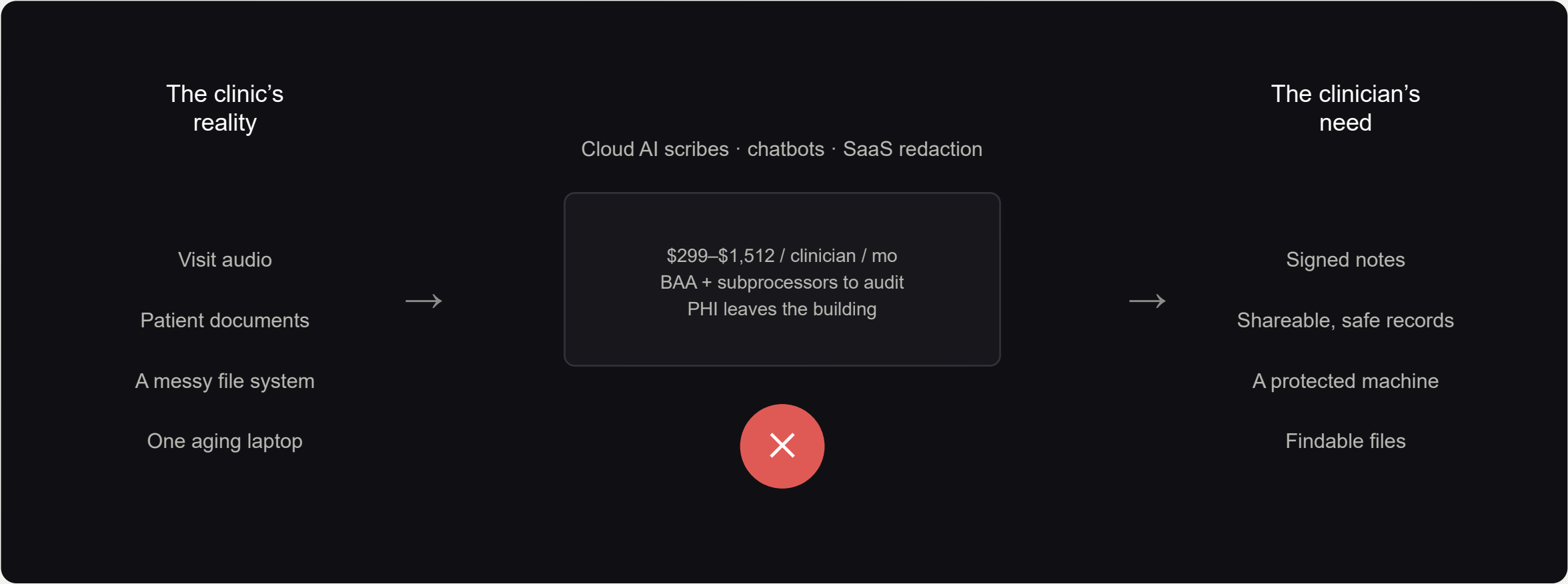
A protected machine

Findable files



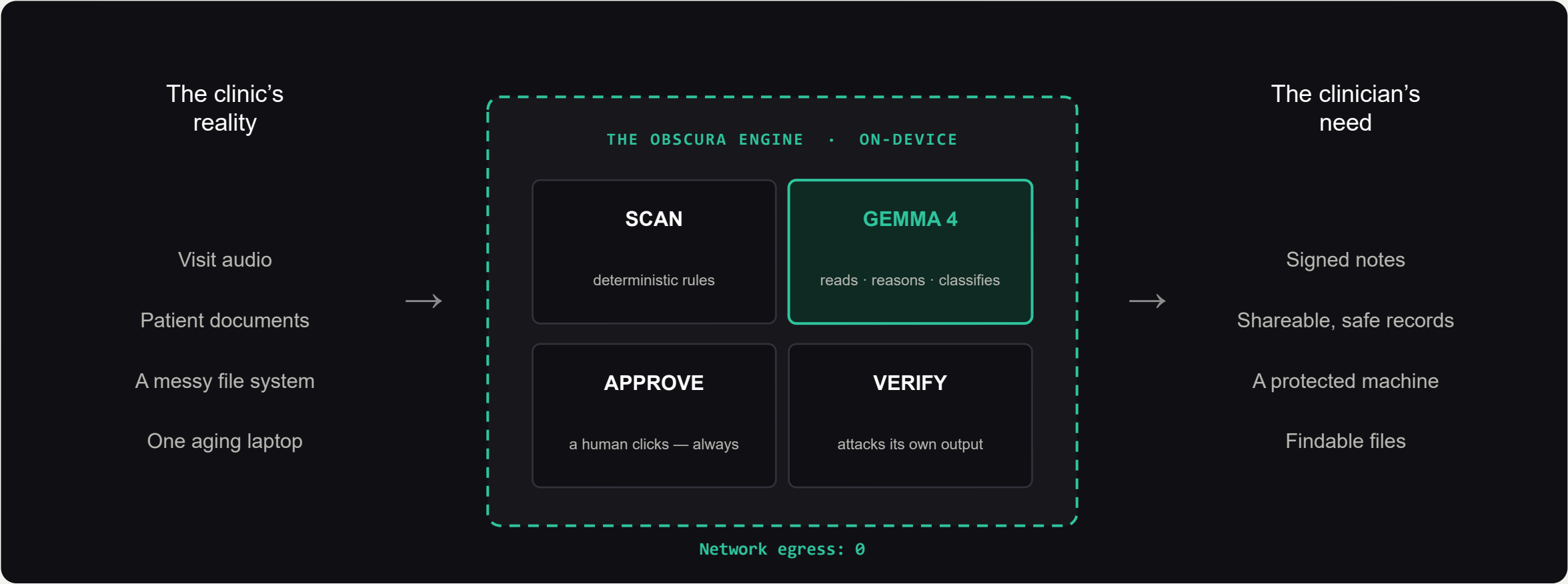
Not through the cloud.

Consumer AI tools won't sign a HIPAA Business Associate Agreement. For a patient record, the upload is the disclosure.



Through one engine that never leaves the room.

Gemma 4 — open weights, Apache-2.0 — runs frontier reasoning on the laptop the clinic already owns.



Obscura →

One engine.
Four surfaces.

01

Transcribe

The visit writes itself.

on-device speech → SOAP draft → clinician signs · no BAA because no third party

IN DESIGN

02

Redact

Share the record, not the patient.

all 18 HIPAA Safe Harbor identifier categories · destroys pixels, never covers text

SHIPPED

03

Secure

A \$0 IT department.

CIS Level 1 checks · plaintext-secret scan · Safety Score · read-only, never executes

SHIPPED

04

Organize

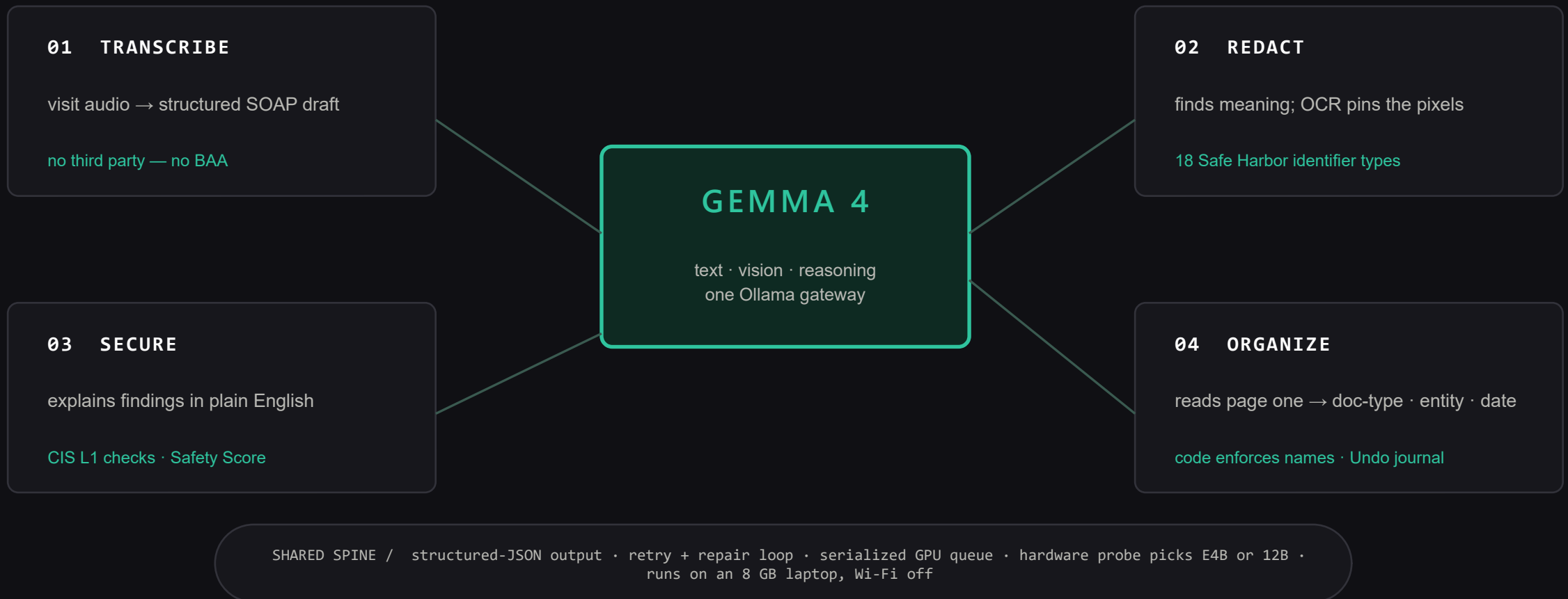
Files that name themselves.

Gemma reads page one · template-enforced names · crash-safe journal · one-click Undo

SHIPPED

The surfaces feed each other: SECURE finds a plaintext key → REDACT destroys it → ORGANIZE files the result.

One local **Gemma 4** model powers every surface — **before, during, and after** each patient touchpoint



Obscura →

Obscura in Action

Built in one day. Verified before it was demoed.

0 recoverable characters after redaction — select-all, text-search, re-OCR, metadata audit all pass

16/18 HIPAA Safe Harbor identifier categories flagged on a synthetic patient record, live coverage panel

4 verification checks the tool runs against its own output on every single export

8 GB of laptop GPU is enough — a hardware probe picks the right Gemma 4 variant per machine

VERIFICATION INDEX

01/	SELECT-ALL TEST	zero selectable characters	PASS
02/	TEXT-SEARCH	no redacted string findable	PASS
03/	RE-OCR	nothing readable survives in pixels	PASS
04/	METADATA AUDIT	document dictionary empty	PASS
05/	EGRESS PANEL	external connections: zero, live	PASS
06/	UNDO REPLAY	full reorganization reversed	PASS

The market charges \$15,000+ a year for what a two-provider clinic gets here at \$0.

AI Scribe

\$299–\$1,512 / clinician / mo

Suki · Nuance DAX Copilot (list \$1,512; typical \$369–830 + \$5k–50k setup) · budget tier Freed \$79–119

Redaction Suite

\$279–\$379 / user / mo

CaseGuard Doc & Ultimate suites · budget tier Redactable from \$278/yr

Managed IT + HIPAA

\$100–\$250 / user / mo

small-practice HIPAA-ready MSP plans · HIPAA support adds \$15–30/user

OBSCURA

\$0 · forever

Apache-2.0, on hardware the clinic already owns — in a \$1.2–2.8B market headed to ~\$15B by 2034–35, all cloud, none priced for small practices

Pricing: vendor list / published reseller pages + 2026 pricing guides; market size: Dataintelo, Astute Analytica, SNS Insider, Grand View 2025–26. List prices — verify per quote.

Obscura →

Getting Started

From zero to private AI in one afternoon.

- 01 Install Ollama · pull Gemma 4 (two commands)
- 02 Clone the repo · pip install · run one server
- 03 Turn the Wi-Fi off — everything still works
- 04 Redact, scan, and organize with a human click on every action

The ask: pilot clinics to shape TRANSCRIBE · open-source contributors · EHR-integration partners

The same four surfaces generalize —

OBSCURA FOR CLINICS

OBSCURA FOR LEGAL AID

OBSCURA FOR SOCIAL WORK

OBSCURA FOR SCHOOLS

OBSCURA FOR FOIA OFFICES

OBSCURA FOR COURTS

OBSCURA FOR SHELTERS

OBSCURA FOR EVERYONE

As fast as AI. As private as a locked filing cabinet.

Open models made this inevitable. We just built it first.

OBSCURA · [GITHUB.COM/MEETKPATEL/OBSCURA](https://github.com/meetkpatel/obscura) · BUILT ON GEMMA 4 · [APACHE-2.0](#)